ImaarishaSRHR

PHASE 2 · MEDIA DEVELOPMENT · COMPANION CURRICULUM

The Mother & Aunty Curriculum

A facilitator's curriculum for turning older women from gatekeepers of harmful SRHR norms into champions of their daughters' health — how patriarchy, culture and disinformation pass down the generations, and how to interrupt that transfer respectfully and effectively.

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Companion to the Strategic Comms & Reframing SRHR field guide
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Built around Beth — a composite Kenyan aunty from the Phase 1 research. Every figure and case is sourced; bracketed numbers [n] refer to the Sources list.

WHY THIS CURRICULUM

The highest-leverage room in Kenyan SRHR

THE STORY · Meet Beth

Beth is fifty-two. She raised three of her own children and half-raised her brother's. She runs the family kitchen, admins the "Achieng Family" WhatsApp group, and is the first person her seventeen-year-old niece Agneta comes to when she is frightened. When Beth speaks about a young woman's body, the family listens — more than they listen to any teacher, nurse or poster.

That is exactly why Beth matters. She is not a barrier to go around; she is the most trusted messenger in Agneta's life. Today she passes on warnings that harm — "family planning will make you barren," "marry by twenty" — not from malice but from love, carrying what she herself received as truth. This curriculum is about one thing: helping the Beths of Kenya carry accurate love instead. When Beth changes, she does not change one girl. She changes every girl who ever helps her cook.

The Phase 1 research names older women as Phase 2's single highest-leverage intervention point. 67% of Kenyans say older women shape younger women's confidence to make SRHR decisions; 64% say they enforce traditional norms; 60% say they decide whether a young woman seeks services openly or in secret. No campaign, clinic or poster reaches into a young woman's life with that authority. The channel that carries the harm is also the only channel powerful enough to carry the correction.[2,1]

67%	64%	60%
older women shape younger women's SRHR confidence	older women enforce traditional norms	they decide open vs secret service-seeking

Older-women influence: TIFA opinion poll[2].

FRAMEWORK · The gatekeeper–champion thesis (S5)

The strategy of this curriculum rests on one evidence-backed idea: older women are **not a fixed obstacle — they are a strategic audience**. The Grandmother Project's work across West Africa shows that when older women are engaged with respect for their cultural role and given accurate information within it, they move from enforcers of harmful norms to their most powerful reformers. The same authority that once defended a practice can be turned to defend a girl's health.[5,6,1]

Who this curriculum is for

It is written for facilitators — community health promoters, peer educators, programme officers, faith and community mobilisers — who will convene and walk groups of older women (and, in places, older and younger women together) through a change process. It assumes no prior training in social-norms theory; the theory is built in as you go. It is designed to be run over five core sessions, weekly or fortnightly, with follow-up.[1]

PART A

How the transfer works

Before you can interrupt the transfer of harmful norms, you have to see it clearly: what is passed down, through which channels, and why it is so hard to counter. This is the diagnostic half of the curriculum — the map facilitators carry in their heads. [1]

What is transferred

Older women do not pass down a single message; they pass down a bundle — patriarchy, culture and SRHR disinformation braided together so tightly they arrive as one piece of advice. Pulling them apart is the first skill. [1,4]

Strand	What it sounds like from Beth	What it really is
Patriarchy	"A woman's worth is her children. Give your husband many." "Let the man decide."	Male reproductive authority, passed woman-to-woman as womanly duty (S4).
Culture & shame	"Good girls wait. Only loose girls go to the clinic." "Don't bring shame on this family."	Injunctive norms enforced by anticipated shame — the shame economy (S5/S3).
SRHR disinformation	"Family planning makes you barren." "Depo damages the womb."	Contraceptive-safety myths (Typology 1), delivered as protective fact (S6).
Fertility pressure	"Start early while your body is strong. Don't wait too long."	Early-marriage / early-childbearing norm dressed as health advice (S5).

Strands and verbatim patterns: focus groups and social listening [4,3].

Through which channels

The transfer is not one conversation; it runs on specific, identifiable channels. Each needs a different interruption (Part C). [1]

Channel	How the transfer happens here	Why it is powerful
Face-to-face — the kitchen	Advice given while cooking, plaiting hair, at the market — the intimate one-to-one where Beth counsels Agneta directly.	Highest trust, total privacy, delivered as love; no third party can rebut it in the moment.
The family WhatsApp group	Beth (admin) and relatives forward "warnings," myths and prayer-wrapped health claims to 30+ kin at once.	Closed and unmoderated; arrives with a trusted relative's endorsement; scales instantly across the family.
Ceremonies & women's gatherings	Weddings, funerals, church women's groups, ruracio and initiation talk where norms are stated publicly and reinforced.	Collective and public; states the injunctive norm ("this is what we approve of") in front of everyone.
The shame economy	Gossip, side-comments and reputation — "people will talk" — that police a girl's visible behaviour.	Enforcement, not information; raises the cost of a girl acting even when she knows the facts.

Transfer channels: research report and focus groups [1,4].

Why it is so hard to counter

FRAMEWORK · **Delivered as love, enforced as a norm**

Two mechanisms make the transfer almost immune to conventional campaigns. First, **it arrives as love**: because the message comes from the person most invested in the girl's wellbeing, it bypasses the critical filtering a stranger's claim would meet. Second, **it is a norm, not an opinion**: Agneta may privately know the facts, but she conforms to what she believes Beth and the family approve of (the injunctive norm) and what she believes other "good" girls do (the descriptive norm). Facts alone do not move a norm — only a shift in what the trusted reference group is seen to do and approve. [7,8,1]

There is also a hard truth the evidence insists on: older women's influence is **bidirectional**. Where a mother-in-law or aunt endorses spacing, contraceptive use rises. The goal is therefore never to remove Beth from the conversation — it is to change what she endorses. Bypass her and the girl's decision goes underground into secrecy; convert her and she protects every girl in her orbit. [9]

PART B

The gatekeeper → champion pathway

Change is not a switch; it is a path with stages. Naming the stages lets a facilitator diagnose where a given aunty is and choose the right move — you cannot ask someone at Stage 0 to do a Stage 3 thing. Here is the pathway, illustrated through Beth. [1]

Stage	What Beth looks like here	What moves her to the next stage
0 · Gatekeeper	Certain and protective; passes on the warnings as settled truth. Not open to a stranger's facts.	A trusted convener; her cultural role honoured, not attacked; an invitation to reflect, not a lecture.
1 · Doubt	Something unsettles her — a clinician she respects, another aunty's story, a granddaughter's pain. "Maybe not everything I was told is true."	Accurate information inside her values; permission to have been misinformed without being blamed.
2 · Reframe	She separates her love (right) from her facts (wrong). "I still want to protect Agneta — but the safe way is different from what I thought."	The new scripts: language that keeps her intention and intimacy but carries accurate content.
3 · Adopt	She quietly changes her own advice in the kitchen and the group; she goes with a girl to the clinic.	Practice and reinforcement; a peer group of aunties doing the same; small early wins she can see.
4 · Champion	She actively persuades other aunties; she tells her own change-of-heart story; she defends a girl's health publicly.	Recognition, a platform, and safeguarding — plus her own story turned into content (Part D, Session 5).

Pathway synthesised from the Grandmother Project evidence and social-norms diffusion theory [5,7,1].

ILLUSTRATION *A five-panel strip showing Beth's face changing across the stages — arms crossed and certain at Stage 0, thoughtful at Doubt, softening at Reframe, walking beside Agneta at Adopt, speaking to a circle of aunties at Champion. Same woman, one journey.*

Two cautions. Progress is not linear — an aunty can slip back under family or church pressure, so reinforcement matters as much as the first shift. And the pathway is a diagnostic, not a script to rush: an aunty pushed from 0 to 4 in one meeting will revert. Meet each woman where she is. [1]

PART C

Interrupting the transfer — respectfully and effectively

Five principles

1 · Honour the intention. Always name the love first. "You want Agneta to have a good future — so do we." The intention is right; only the information is wrong.

2 · Never shame the messenger. Attacking Beth's beliefs attacks Beth. She will defend the old norm harder, and you will lose the channel. Shame is the one thing that reliably fails.

3 · Work through her, not around her. Do not bypass Beth to reach Agneta directly — that drives the decision into secrecy. Change what Beth endorses and you change the whole family.

4 · Keep the love, change the facts. The new script must still sound like a caring aunty, not a campaign. Do not take the warmth out of the sentence — swap the false fact inside it.

5 · Give her a role, not just a correction. People defend what they help build. Move Beth from audience to author — of the new scripts, and of her own story.

Interrupting, channel by channel

Each transfer channel from Part A needs its own interruption. This is the operational heart of the curriculum. [1]

In the kitchen (face-to-face)

This is the hardest channel to reach because it is private — which is why the intervention is Beth herself, retrained. The method is the "rewrite the warning": take her real words and rebuild them so they keep the intimacy and the intention but carry the truth. [10]

Beth's old kitchen script	Beth's new kitchen script (same love, true facts)
"Family planning will make you barren — wait for marriage."	"I want you to have healthy children when you're ready. Spacing your births is how strong mothers plan — it doesn't stop you having children later. Let's ask Nurse Charity together."
"Only loose girls go to the clinic alone."	"A girl who looks after her health is a responsible girl. If you ever want to go, I'll come with you — no shame in it."
"Marry by twenty and start early."	"Finish your training first — a nurse with her own income is a strong woman. Children will come when you and your husband are ready."

In the family WhatsApp group

Because the group is closed and Beth is the admin, no outside correction can enter — only Beth's can. So the intervention trains Beth to be the group's calm corrector, using the truth-sandwich structure (fact → myth once → why it spreads → fact) in a short, forwardable message. [10]

> **A message Beth can post in her own group**
>
> *"My people, I checked this one with the nurse. Family planning does NOT make a woman barren — your*

> *fertility returns after you stop. I know we all heard otherwise from people who love us, but it isn't true. Let's protect our girls with the right information. 🙏"*

- **Recruit the admins.** The person who can correct a family group is the person who runs it. Map and train the Beths.
- **Give them ready assets.** Supply forwardable Kiswahili voice notes and images so correcting is one tap, not an essay.
- **Never quote the myth louder than the fact.** Lead with the truth; state the myth once; do not amplify it.

At ceremonies & women's gatherings

Here the norm is stated publicly, so the interruption must also be public and carried by peers. A champion aunty who has reached Stage 4 stating a new norm at a women's group — "these days we let our girls finish school first" — resets what the room believes is approved. This is diffusion of innovations: a respected insider gives everyone else permission to follow.[5]

At the clinic and the doorstep

Community health promoters are the bridge into the kitchen the programme itself cannot enter. Brief every CHP on the pathway and the new scripts so that when they visit the home, they reinforce — not contradict — the change Beth is making.[1]

ENTRY POINTS FOR YOUR ORGANISATION · Wiring the interruption into your programme

→ Map the champion aunties and family-group admins in your catchment — they are your delivery network.

→ Produce the new-script kitchen cards and WhatsApp voice notes in Kiswahili and local languages.

→ Brief every CHP and mobiliser on the gatekeeper→champion pathway so home visits reinforce it.

→ Create a public platform (radio, women's groups) for Stage-4 champions to state the new norm.

PART D

The five-session curriculum

This is the facilitated programme itself: five sessions that walk a group of 10–15 older women along the pathway from gatekeeper to champion. Run weekly or fortnightly, ideally in a familiar community space, with tea. Keep groups small enough for everyone to speak. A clinician (a Nurse Charity) co-facilitates Sessions 2 and 5. [1]

SESSION 1

S1 Our role, and our love

Field	Detail
Duration	2 hours
Participants	10–15 older women (mothers, aunties, mothers-in-law, grandmothers)
Objectives	Establish trust; honour the participants' role; surface (without judging) the advice they currently give.
Materials	Tea; name cards; flip chart; the "warnings we give" cards

Facilitator flow

1. Welcome and tea. Open by honouring their role: they are the most trusted advisors in their families. This is not a class that will correct them; it is a conversation among respected women.
2. Round: "What did your mother or aunty tell you about your body when you were young?" Let the stories flow; write the recurring warnings on the flip chart without comment.
3. Reflect together: which of these are we still passing on? Not "are they wrong" — just "what do we say?"
4. Close on the shared value: everything on this chart was said out of love, to protect a girl. Hold that. Next time we will look, together, at which parts are still true.

> ***Say:*** *"None of us is here because we did anything wrong. We are here because our girls listen to us more than to anyone — and that is a gift. Let us use it well, together."*

Take-home. Each woman notices, over the week, one piece of advice she hears an aunty give a young woman — and brings it back.

SESSION 2

Learning activity — The advice we carry

Surface, without judgement, the advice participants currently pass on — so the room owns the material before any of it is challenged.

⍰ 20 min ⍰ Individual → plenary

You'll need: Sticky notes or small cards; one wall

1. On a card, each woman writes one piece of advice she gives — or was given — about a girl's body, marriage or children. No names.
2. Post every card on the wall, unsigned. Read them aloud without comment.
3. As a room, cluster the cards: which come from love? (All of them.) Which carry a fact we should check?
4. Leave the wall up for the whole course — we will come back to it.

Debrief: What did it feel like to see our own advice on the wall, side by side — and not one of us a bad person for it?

S2 What we were told, and what is true

Field	Detail
Duration	2.5 hours
Participants	The group + a trusted clinician (Nurse Charity) co-facilitating
Objectives	Separate the love (right) from the facts (wrong); replace myths with accurate information from a trusted clinician — without shame.
Materials	The flip-chart warnings from Session 1; the Myth & Fact elder card (Appendix); a clinician

Facilitator flow

1. Recap the warnings chart. Frame today gently: “We were told these things by people who loved us. Some were true. Some, it turns out, were not — through no fault of ours.”
2. Take the top three myths one at a time. The clinician answers each with the fact and a causal explanation (fertility returns; the womb is not damaged), warmly, never mocking.
3. Use the truth-sandwich shape aloud so the women hear the pattern: the fact first, the myth named once, why it spread (“delivered as love”), the fact again.
4. Open floor: “What surprised you? What do you still doubt?” Let the clinician address doubts without defensiveness.

> ***Say:*** *“If you passed on a warning that wasn’t true, you were not foolish — you were passing on what you were given. Now we have better information to give. That is all that has changed.”*

Take-home. The Myth & Fact elder card to keep. Each woman picks one myth she will stop repeating.

SESSION 3

Learning activity — Love is right, the fact may be wrong

With a trusted clinician, separate the loving intent (keep it) from the inaccurate fact (replace it) — without shame.
⏱ 30 min 👥 Plenary with clinician → individual
You'll need: The Myth & Fact card; the wall from Session 1

1. Take three cards off the Session 1 wall that carry a health claim.
2. For each, name the love in it first; then the clinician gives the accurate fact, kindly.
3. Each woman writes down one myth she now knows is false — and will stop repeating.
4. Keep the Myth & Fact card; it goes home.

Debrief: Which fact surprised you most — and which will be hardest to stop passing on, even now that you know?

S3 The new scripts

Field	Detail
Duration	2.5 hours
Participants	The group
Objectives	Turn the corrected facts into language that still sounds like a caring aunty — the "rewrite the warning" skill.
Materials	The old/new script sheet; blank cards; pens

Facilitator flow

1. Introduce the rule: keep the love, change the facts. A good new script still sounds like you, not like a poster.
2. Model one together on the flip chart, from a real warning in the group's own words.
3. In pairs, each woman rewrites two of her own warnings into new scripts. Facilitator circulates and helps.
4. Role-play in the round — aunty to "niece" — until the new script sounds natural and warm. The room gives feedback on believability, not correctness.

> ***Say:*** *"Do not take the love out of the sentence. Keep the love. Change only the facts inside it."*

Take-home. Each woman leaves with two new scripts in her own words, written on cards, to use in her own kitchen.

SESSION 4

Learning activity — Rewrite your own warning

Turn a corrected fact into language that still sounds like a caring aunty — each woman rewrites her own warnings.
30 min **Model together → pairs**
You'll need: Blank cards; the 'rewrite the warning' example

1. As a room, rewrite one warning together: keep the love, name the myth once, offer a new line.
2. In pairs, each woman rewrites two of her own warnings from the Session 1 wall.
3. Say the new line aloud, in your own voice — would a niece hear love, or a lecture?
4. Write the two best on cards to carry.

Debrief: What had to stay the same for it to still sound like you — and what had to change?

S4 Carrying it — the kitchen and the phone

Field	Detail
Duration	2 hours
Participants	The group
Objectives	Practise using the new scripts in the two channels that matter most: the face-to-face conversation and the family WhatsApp group.
Materials	Phones; the forwardable voice-note and image assets; scenario cards

Facilitator flow

1. Kitchen practice: scenario cards ("your niece says she is scared of getting pregnant and ruining school") — women respond aloud using their new scripts.
2. WhatsApp practice: show the ready-made correction message and voice notes. Each woman who is a group admin practises posting or forwarding one.
3. Discuss the hard moments: what if the men in the group push back? What if the pastor said otherwise? Problem-solve together.
4. Agree a simple commitment each woman will make in her own family this week.

Take-home. Each woman uses one new script in a real kitchen conversation and forwards one accurate voice note to her family group.

SESSION 5

Learning activity — Two rooms, two scripts

Practise the new scripts in the two channels that matter most — the face-to-face talk and the family WhatsApp group — and problem-solve pushback.

30 min **Role-play in threes**

You'll need: A phone (for the WhatsApp role-play); a ready-made correction voice note

1. In threes, role-play a kitchen talk: one aunty, one niece, one observer. Use a new script.
2. Swap: now role-play the family WhatsApp group — an admin forwards an accurate voice note after a bad forward.
3. The observer names one thing that worked and one pushback (from a man, the pastor); the group solves it.
4. Each admin saves one correction voice note to forward for real.

Debrief: Which room is harder for you — the kitchen or the group — and what makes it harder?

S5 Becoming a champion

Field	Detail
Duration	2.5 hours
Participants	The group + clinician + programme lead
Objectives	Move willing women from private change to public championing; capture change-of-heart stories; set up safeguarding and ongoing support.
Materials	Recording consent forms; the champion pledge card; refreshments

Facilitator flow

1. Round: “What changed for you, and what did you do differently?” These are the change-of-heart stories — the most persuasive content the programme has.
2. With consent, capture two or three short stories (audio or on camera) for use on radio, WhatsApp and the podcast — safeguarded and voluntary.
3. Introduce the champion role: speaking up in women’s groups, mentoring one or two younger aunties, going with a girl to the clinic when asked.
4. Set up ongoing support: a champions’ WhatsApp group, a monthly check-in, and a named programme contact. End with the pledge and celebration.

Safeguarding note: championing is voluntary and must never expose a woman to family or community backlash she has not chosen. Capture and share stories only with informed, ongoing consent; a woman may withdraw at any time. Protect the champions as carefully as the girls they now protect. [12]

PART E

Learning activity — The pledge and the story

Move willing women from private change to public championing — capture a change-of-heart story and set up ongoing support.

⍰ 35 min ⍰ Plenary → consenting volunteers

You'll need: A phone to record; the Champion Aunty card; a consent line read aloud

1. Invite — never pressure — two or three women to record a short change-of-heart story for radio / WhatsApp / podcast, with clear spoken consent.
2. Introduce the Champion Aunty role and card; each champion names one younger aunty she will mentor.
3. Set up the champions' WhatsApp group and a monthly check-in; name the first date now.
4. Close with the pledge and a celebration.

Debrief: What will help you keep this going when the course is over and the room is gone?

Facilitator's toolkit

Recruiting the right champions

Not every older woman is a champion in waiting. Recruit for standing and openness, not seniority alone.[5]

- **Standing.** Respected in her network — an opinion leader others already watch (a family-group admin, a women's-group elder).
- **Openness.** Shows some willingness to reconsider — asks questions, mentions a granddaughter, is troubled by a girl's pain.
- **Reach.** Sits at a busy junction — many nieces, a large family group, a market stall where women gather.
- **A story.** Ideally has her own change-of-heart to tell; nothing persuades another aunty like one of their own who changed.

Do and don't

Do	Don't
Open every session by honouring their role and their love.	Never open with what they are doing wrong.
Let a clinician carry the facts; let the women carry the language.	Don't let a young facilitator "correct" elders — it inverts the respect the method needs.
Meet each woman at her stage; reinforce small shifts.	Don't rush a gatekeeper to champion in one meeting.
Keep the new scripts warm and in local language.	Don't hand out poster-language slogans to say in a kitchen.

Measuring change

Track movement along the pathway, not just attendance. Simple, honest markers:[1]

- Aunties can state, in their own words, at least two accurate new scripts (Stage 2–3).
- Aunties report using a new script in a real family conversation (Stage 3).
- Family-group admins post or forward at least one accurate correction (Stage 3).
- At least some aunties speak up in a women's group or mentor another aunty (Stage 4).

- Change-of-heart stories are captured (with consent) and travelling on radio/WhatsApp (Stage 4).

APPENDIX A

The Champion Aunty’s card (template)

A pocket card for trained champions — the pledge on one side, the new scripts on the other.

MY PLEDGE

I am a trusted voice in my family. I will pass on love AND the right facts. I will not shame a girl for looking after her health. When a girl comes to me afraid, I will listen, tell her the truth, and go with her to the nurse if she wants. My daughters’ health is my honour.

APPENDIX B

Myth & Fact card for elders

The elder-facing quick reference used in Session 2. Lead with the fact; name the myth once.

The warning we heard	The truth to pass on instead
Family planning makes a woman barren.	Fertility returns after you stop. Spacing births does not stop you having children later.
Depo / the injection damages the womb.	These methods are safe and well studied. If one doesn't suit her, the nurse can change it.
Contraception means a girl plans to be loose.	Looking after your health is responsible. Many married women space their births too.
A girl must marry and start children early.	A girl who finishes school and trains is a strong woman; children come when she is ready.
Going to the clinic brings shame.	A girl who cares for her health is to be respected. Go with her if she is afraid.

Facts confirmed with clinical guidance; belief prevalence: TIFA poll [2].

APPENDIX C

WhatsApp voice-note scripts for champions

Short, warm, forwardable — for champions to record in their own voice, in Kiswahili or the local language.

1. On fertility: “My people, I asked the nurse myself. Family planning does not make our girls barren — their fertility comes back. Let us stop scaring them and start protecting them with the truth.”
2. On school first: “A girl who finishes her training and earns her own money is a blessing to the whole family. Let our daughters finish first. The children will come at the right time.”
3. On going together: “If your daughter or niece is afraid to go to the clinic, go with her. There is no shame in a woman caring for her health.”
4. On a spreading myth: “Before you forward that warning — ask the nurse first. Many things we were told were not true. Let us not pass fear to our girls.”

SOURCES

Sources & References

Sources [1]–[4] are the ImaarishaSRHR Phase 1 evidence base. Public and academic sources follow, in order of first appearance.

[1] CoMeT / ImaarishaSRHR, "Deconstructing Cultural & Patriarchal Barriers to SRHR Access and Enjoyment in Kenya," triangulated mixed-methods research report, March 2026.

[2] TIFA Research, "How Patriarchy, Culture & Disinformation Influence SRHR Access — Opinion Poll" (n=2,013, all 47 counties, ±2.18%), May 2026.

[3] ImaarishaSRHR Social Listening dataset — 419,720 posts (Jul 2025–Feb 2026).

[4] ImaarishaSRHR WhatsApp Focus Group Discussions — women 18–35, 4 sites, 2025–2026.

[5] Aubel J., "Grandmothers — a neglected family resource for saving newborn lives," BMJ Global Health 6:e003808, 2021. *https://pmc.ncbi.nlm.nih.gov/articles/PMC7887373/*

[6] Aubel J., "Grandmothers — a cultural resource for women and children's health and well-being across the life cycle," Global Health Promotion, 2024. *https://journals.sagepub.com/doi/10.1177/17579759231191494*

[7] Bicchieri C., "Norms in the Wild: How to Diagnose, Measure, and Change Social Norms," Oxford University Press, 2017.

[8] Institute for Reproductive Health, Georgetown University / Learning Collaborative to Advance Normative Change, "Social Norms Lexicon," 2021. *https://www.irh.org/wp-content/uploads/2021/05/Social-Norms-Atlas.pdf*

[9] Bhan N. et al., "Mother-in-law's Influence on FP Decision-Making and Contraceptive Use: A Review of Evidence," Center on Gender Equity & Health (UCSD) & ICRW, 2022. *https://emerge.ucsd.edu/wp-content/uploads/2022/08/mil-brief_geh_geh_icrw.pdf*

[10] Lewandowsky S., Cook J. et al., "The Debunking Handbook 2020." *https://digitalcommons.unl.edu/context/scholcom/article/1247/viewcontent/DebunkingHandbook2020.pdf*

[11] The Challenge Initiative, "Intergenerational Dialogue" demand-generation curriculum. *https://tciurbanhealth.org/courses/nigeria-demand-generation/lessons/intergenerational-dialogue/*

[12] Dichter M.E. et al., "Survivor storytelling for social change," Journal of Interpersonal Violence, 2022. *https://doi.org/10.1177/10778012211024267*

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